

TB-12 ABDOMINAL / PELVIC TRAUMA

Trauma & Burns | EMT - AEMT - Paramedic

Core principle: The abdomen, retroperitoneum, and pelvis can conceal life-threatening hemorrhage before hypotension appears. Control external bleeding, stabilize a suspected pelvic fracture, prevent hypothermia, and move rapidly toward definitive hemorrhage control.

QUICK FLOW

1. XABCDE + RAPID SURVEY

Hemorrhage, airway/ventilation, perfusion, neuro status, expose and warm.

2. INTERNAL HEMORRHAGE?

Shock, tenderness, distention, seatbelt sign, flank bruising, high-risk mechanism.

3. OPEN INJURY?

Do not remove an object or reduce evisceration; stabilize and cover.

4. PELVIC INJURY?

Mechanism, pain/deformity, shock; binder over greater trochanters.

5. RESUSCITATE EN ROUTE

IV/IO, warming, analgesia/antiemetic, shock pathway; blood/TXA if adopted.

6. TRAUMA CENTER

Minimize scene time; serial exam/vitals; pregnancy/pediatric considerations.

PROVIDER-LEVEL ACTIONS

EMT	Perform XABCDE, control external hemorrhage, prevent hypothermia, and rapidly assess the abdomen/flanks/pelvis. Do not repeatedly rock the pelvis. Apply the approved binder over the greater trochanters when indicated. Do not remove impaled objects or reduce evisceration. Package for rapid trauma transport.
AEMT	All EMT actions plus IV/IO, warmed crystalloid only under the traumatic-shock pathway, and authorized analgesia/antiemetic care. Do not delay transport for access; avoid unnecessary large-volume crystalloid.
PARAMEDIC	All prior actions; lead airway/ventilation, monitoring, shock resuscitation, Medical Control, destination, and air-medical decisions. Blood products and TXA require a separately finalized Claiborne standing order.

RAPID ASSESSMENT

- Mechanism, energy transfer, penetration trajectory, restraint/seatbelt, impalement, anticoagulants, pregnancy/gestation, and associated chest, spine, head, or extremity injury.
- Inspect and gently palpate once for pain, guarding, rigidity, distention, abrasions, ecchymosis, penetrating wounds, evisceration, or exposed organs.
- Inspect the pelvis, groin, and perineum for wounds, bruising, deformity, swelling, blood at the urinary meatus, hematuria, rectal/vaginal bleeding, or neurologic deficit while preserving privacy and warmth.
- Trend mental status, pulse, skin, capillary refill, blood pressure, pulse pressure, respiratory status, and response to care. A normal initial pressure does not exclude compensated hemorrhage.

HIGH-RISK FINDINGS

Urgent trauma-center transport / early alert: shock; penetrating torso injury; evisceration or impalement; expanding abdomen/flank/groin hematoma; pelvic deformity or pain after high-energy trauma; seatbelt sign; guarding/rigidity; hematemesis; hematuria or blood at the meatus; pregnancy; pediatric/older patient; anticoagulant use; or unreliable examination.

PELVIC STABILIZATION

- Use mechanism, pain, deformity, and shock to identify a suspected pelvic fracture. Do not repeatedly rock or compress the pelvis to test stability.
- Apply the agency-approved commercial binder centered over the greater trochanters, not the abdomen, waist, or iliac crests. Secure the legs together when appropriate and reassess distal circulation, sensation, and movement.
- Minimize movement. Do not loosen or remove a correctly positioned binder in the field unless a separately approved protocol or Medical Control requires it.
- Use only an appropriately sized, agency-approved pediatric device and application procedure.

EVISCEARATION / IMPALED OBJECT

- Evisceration: do not replace organs. Cover loosely with sterile saline-moistened dressings and an occlusive layer when available. Avoid direct pressure and protect from heat loss.
- Impaled object: do not remove. Stabilize in place with bulky dressings without pressure on exposed organs. Modify/remove only for an immediate lifesaving intervention or safe transport under rescue/Medical Control coordination.
- Use direct pressure, wound packing, hemostatic gauze, or tourniquet for compressible external or junctional hemorrhage when anatomically appropriate. Do not blindly pack the abdominal cavity.
- Keep the patient NPO. Do not insert a urinary catheter in the field when blood at the meatus or urethral injury is suspected.

RESUSCITATION + TRANSPORT

- Treat suspected internal hemorrhage as time-critical. Minimize scene time and complete nonessential assessment/procedures during transport.
- Establish IV/IO without delaying transport. Use warmed crystalloid, blood products, TXA, and other shock interventions only under current finalized Claiborne standing orders. Avoid unnecessary large-volume crystalloid.
- Provide authorized analgesia and antiemetic care, prevent hypothermia, reassess frequently, and notify the trauma center early.
- Use the current Tennessee trauma destination criteria. Consider air medical only when expected to reduce time to definitive care and without delaying ground movement.

PREGNANCY / PEDIATRIC

PREGNANCY	Prioritize maternal resuscitation. Beyond approximately 20 weeks, use manual left uterine displacement or left lateral tilt when compatible with spinal/trauma care. Notify a trauma/obstetric-capable destination.
PEDIATRIC	Use age-appropriate shock signs and the length-based system. Hypotension is late. Do not delay hemorrhage control, binder placement when indicated, or transport.

DOCUMENT

- Mechanism, serial examination/vitals, suspected bleeding source, binder indication/position/time, distal neurovascular findings, open-injury care, medications/fluids/blood, response, pregnancy status, consultation, and destination rationale.

DRAFT FOR REVIEW. Confirm binder model/sizing/training, evisceration supplies, hemorrhagic-shock pathway, blood/TXA policy, analgesic/antiemetic formulary, and trauma/obstetric destination plan before clinical release.

Reference: Tennessee EMS ALS/BLS Blended Protocol Guidelines 2024-2025, SOP #402 Abdominal / Pelvic Trauma, Hypovolemic Shock, and Trauma Assessment / Destination Guidelines. Approved Claiborne standing orders control.